

GOUT

- An acute gout attack tends to affect a single joint, most commonly the big toe or knee. There is a sudden onset of severe pain, redness and swelling. It resolves completely, usually within days.
- Chronic tophaceous gout tends to asymmetrically affect > 1 joint and may not be very painful. Deposits can be seen or felt at the joints and there is incomplete recovery.

Assess the patient with gout

Assess	When to assess	Note
Symptoms	Every visit	Manage symptoms as per symptom pages.
Alcohol/drug use	Every visit	In the past year, has patient: 1) drunk ≥ 4 drinks ¹ /session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ➔ 142.
Medication	Every visit	Hydrochlorothiazide, furosemide, ethambutol, pyrazinamide and aspirin may induce a gout attack. Discuss with doctor. Avoid stopping aspirin given for CVD risk.
Joints	Every visit	• Recognise the acute gout attack: Sudden onset of 1-3 hot, extremely painful, swollen joints with red, shiny overlying skin (often big toe, knee or ankle). • Recognise chronic tophaceous gout: deposits appear as painless yellow hard irregular lumps around the joints (picture).
CVD risk	At diagnosis, then depending on risk	• Assess CVD risk ➔ 127.
Creatinine (eGFR)	At diagnosis, then 6 monthly	If eGFR < 60, refer.
Urate	• At diagnosis • On allopurinol	• Wait at least 2 weeks after an acute gout attack before checking urate level. If urate > 0.5, start allopurinol (see below). • If starting/on allopurinol: repeat urate monthly and increase allopurinol dose if needed until urate < 0.35, then repeat urate yearly.

Advise the patient with gout

- Help the patient to manage his/her CVD risk ➔ 129. If BMI² > 25 advise to reduce weight.
- Give dietary advice:
 - Reduce alcohol (especially beer), sweetened fizzy drinks, seafood, offal and meat intake.
 - Increase low-fat dairy intake.
 - Avoid fasting and dehydration as they may increase the risk of an acute gout attack.
- Advise patient to avoid medication above that may induce an acute gout attack. Discuss with doctor before starting any new medication.



© Stellenbosch University

Treat the patient with gout

Treat the patient with an acute gout attack

- Give **ibuprofen**³ 400mg with food 8 hourly until pain and swelling are better.
- If peptic ulcer, asthma, hypertension, heart failure or kidney disease, avoid ibuprofen and give instead **prednisone** 40mg daily for 5 days.
- If patient is already using allopurinol, avoid stopping it during the acute attack.

Treat the patient with chronic gout

- Patient needs allopurinol if any of: ≥ 2 attacks per year, chronic tophaceous gout (picture), kidney stones, kidney disease, serum urate > 0.5.
- Wait at least 3 weeks after an acute gout attack before starting allopurinol.
- Start **allopurinol** 100mg (or 50mg if ≥ 65 years) daily. Use lowest dose to keep urate < 0.35: if needed, increase monthly by 100mg daily, up to 400mg daily in divided doses. Usual maintenance dose 300mg daily. For doses > 300mg, divide dose.

- If no response to treatment or unsure about diagnosis, doctor to discuss/refer patient to specialist.
- If patient < 40 years or has BMI² < 18.5, refer within 1 month to exclude possible cancer cause for gout.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ²BMI = weight (kg) $\div$ height (m) $\div$ height (m). ³If patient also taking aspirin, advise to wait at least 30 minutes after taking aspirin before taking ibuprofen.